



POLICY AND PROCEDURE

Medication: Prescribing, Administration and Management for Inpatients

Scope (Staff):	All NMHS Mental Health (MH) Inpatient Clinical Staff
Scope (Area):	NMHS MH Areas: Inpatient including Hospital in The Home (HiTH)

Contents

1. Aim.....	2
2. Background.....	2
3. Risk.....	2
4. Definitions	2
5. Principles	3
5.1 Adverse Drug Reactions (ADR) and Allergies	3
5.2 Authorised Medication Charts	4
5.3 Prescribing of Medications	4
5.4 Telephone Orders.....	6
5.5 Administration of Medication	7
5.6 Refusal of Medication	9
6. Error Management.....	9
7. Consumer Medication Information (CMI)	9
8. Nicotine Replacement Therapy (NRT)	10
9. Medication for Hospital in The Home (HiTH) Patients.....	10
10. Storage and Security of Medications.....	11
11. Patient's Own Medications (POMs).....	11
12. Controlled Medicines.....	12
13. Legislative Requirements.....	12
14. Compliance and Evaluation	12
Appendix 1: Acceptable Prescribing Terms and Abbreviations.....	15
Appendix 2: Medications which may be administered by a Registered nurse without a prescription	16
Appendix 3: Medication Dispensed from Ward Stock	17
Appendix 4: Charting Medicines with Generic &/or Trade/ Brand Name.....	19

1. Aim

To provide information and guidance on the storage, ordering, supply and administration of medications. This refers to all disciplines that are authorised to prescribe, dispense, administer or manage medications.

2. Background

This policy document will be followed in accordance with the [Western Australian Legislation - Medicines and Poisons Act 2014](#), [Western Australian Legislation - Medicines and Poisons Regulations 2016](#), [WA Health policy: Reporting of Schedule 4 and Schedule 8 Medicines Discrepancies Policy \(MP 0103/19\)](#), [WA Medicines Handling Policy \(MP 0139/20\)](#), [WA Statewide Medicines Formulary \(Formulary One\)](#) and the [WA Health Credentialing and Defining the Scope of Clinical Practice Policy MP 0084/18](#).

This policy must be read in conjunction with NMHS overarching policy documents:

- [NMHS Administration of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy Procedure](#).
- [NMHS MH Medication: Formulary and Non-formulary Prescribing Policy](#).
- [NMHS MH Medication: Management of Controlled \(Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\)\) Medicines](#).

3. Risk

Non-compliance with this policy may result in a breach of legislative and mandatory requirements, harm to patients, staff or others, and/or reputation and image damage to NMHS.

Staff Safety & Wellness	☒	Policy & Legislative Compliance	☒
Governance & Accountability	☒	Quality of Patient Care	☒

4. Definitions

Administration	The preparation and giving of a medicine to a patient by any route, usually for the patient to take immediately.
Approved Staff Member	Refer to a NMHS staff member who is: • an authorised staff member, or • a staff member who is allocated the responsibility to complete a particular task related to medicines as per their job description.
Chain of Custody	In relation to medicines, the chronological documentation that records the transfer of a medicine from one individual/area to another. Sometimes described as the “paper trail”, even though the records may be electronically made and stored. A critical element of maintaining the chain of custody is that the records must be robust and auditable.
Controlled Substance	This term includes both Schedule 8 (S8) and (drugs of addiction; drugs of dependence) Schedule 4 Restricted (S4R) Medicines (considered to have a higher risk of diversion).

Delegation	Within the context of nursing is the action by which a RN/Midwife delegates aspects of care to another care provider who has the appropriate education, knowledge & skills to undertake the activity within their scope of practice safely, within their scope of practice.
Dispense	In relation to a medicine means supplying the medicine in accordance with a prescription duly given by a Medical Practitioner. Only a Pharmacist or Medical Practitioner may dispense medicines.
Diversion	Unauthorised possession, supply, or receipt of a scheduled medicine for misuse, abuse or illicit use.
Enrolled Nurse	An Enrolled Nurse is defined as a Nurse who is registered with the Australian Health Practitioner Regulation Agency (AHPRA) as an enrolled nurse in Division 2 of the Register.
Medication	Defined as any substance or medication used for therapeutic or diagnostic purposes.
Student Nurse	This refers to a student undertaking preparation to practice as a Nurse (EN or RN). A Student nurse may administer, transport and discard S8 and S4R medicines under the direct supervision of a RN depending on their stage of education and level of competency; only the RN is authorised to sign the medication chart (Refer to NMHS Appendix 1)
Supervision	Defined as Direct Supervision – when the supervisor is present and personally observes, works with, guides, and directs the person who is being supervised.

5. Principles

5.1 Adverse Drug Reactions (ADR) and Allergies

The prescriber must confirm ADR/allergies status prior to prescribing; nurses must check prior to administering medication and pharmacists prior to dispensing.

Any adverse medication reaction must be managed in accordance with the [WA Clinical Alert \(MedAlert\) Policy](#). It must be reported in the patient's Integrated Progress Notes, written on the medication chart, an "Adverse Drug Reaction" sticker must be affixed to the relevant areas of the medication chart, documented in the patient's PSOLIS Alerts, where appropriate, and on the Clinical Alert PMR01 in the front of the patient's clinical records and included within clinical handover.

New ADR/allergies to a medication occurring during an inpatient admission must be reported to the Therapeutic Goods Administration (TGA). Refer to the [TGA On-line Adverse Event](#). The team clinical pharmacist must also be informed.

A Clinical Incident Form (Datix CIMS) and a Clinical Alert Notification form PMR02 should be completed, if the reaction is life-threatening.

All ADRs/allergies must be documented in the patient's discharge summary.

Patient's own adrenaline (epinephrine) autoinjectors (e.g. EpiPen®, Anapen®)



These must be labelled with an addressograph and kept on the ward and are only to be supplied to the patient when they leave the ward for ground access, leave and discharge from hospital.

For treatment of anaphylaxis on the ward, a Code Blue Medical Emergency must be triggered, and the Emergency Drug Box adrenaline should be used.

[Refer also to NMHS MH Management of Clinical Alerts Policy and NMHS Recognising and Responding to Acute Deterioration Procedure](#)

5.2 Authorised Medication Charts

The medication chart is the official record of all medications prescribed and administered to patients. Only medication charts approved by the Quality Improvement and Change Management unit (QICM) can be used.

Use of the WA Hospital Medication Chart – Long Stay (WA HMC) (PMR 60A) with VTE risk assessment documentation is mandatory for regular medication. Guidelines on the use of the WA HMC can be found at [WA Health Guidelines for the WA Hospital Medication Chart](#) (WA HMC Guidelines) and on the Australian Commission on Safety and Quality in Health Care website.

Other medication charts in use are the WA Adult Clozapine Initiation and Titration chart (PMR 60G); Intramuscular Long-Acting Injection chart (Depot Antipsychotic) (PMR 58); WA Adult Anticoagulant Medication chart (PMR 60C); WA Insulin Subcutaneous Order and Blood Glucose Record – Adult Chart and the Agitation and Arousal PRN chart (PMR 60D).

Note:

- ‘PRN’ medications for agitation and arousal must be ordered on the Agitation and Arousal PRN chart (PMR 60D)
- ‘PRN’ medication other than for agitation and arousal must be ordered on the back of the WA HMC (PMR 60A).

5.3 Prescribing of Medications

It is the responsibility of the Medical Practitioner to provide a valid prescription for all medications prior to administration by the nurse. A valid prescription must be legible and meet the requirements of the [Western Australian Legislation - Medicines and Poisons Act 2014](#).

Requirements for prescriptions:

1. Date of prescription.
2. Name, address and date of birth of patient (patient identification labels).
3. Medicine generic name (refer to [Appendix 4 Charting Medicines with Generic / Trade Names](#)), strength, form (tablet, capsule, eye/ear drops etc.), dose.
4. Precise directions for medicine.
5. The printed name and signature of the prescribing Medical Practitioner, and their contact or pager number.
6. Prescribing times and abbreviations are used in accordance with list of acceptable times and abbreviations in Appendix 1.



In addition, medication charts require:

1. The first prescriber **must** print the patient's name in the relevant sections (front and back) of the medication chart and check that the identification label is correct (Refer to [NMHS Patient Identification and Procedure Matching Policy](#) and [NMHS MH Identification of a Patient Procedure](#)).
2. ADR/allergies documented including details of the precise nature of the ADR/allergy, with associated 'Adverse Drug Reaction' sticker.
3. Dose of medication:
 - a. Must be expressed in grams, milligrams, or micrograms. In the case of liquid medicines, the strength and the dose in milligrams or micrograms (not millilitres) must always be specified (e.g., morphine mixture (10mg/mL) Give 10mg every 8 hours).
 - b. The only exception is when a combination tablet or capsule is prescribed, it may be ordered by number of dose units (e.g., 1 tablet); the strengths of the medication in the combination must still be documented:
 - i. The ward/clinical pharmacist must clarify when the strength supplied is different from that ordered. E.g., for 10mg, the pharmacist may write 2 x 5mg tablets or for 25mg, the pharmacist may write ½ x 50mg.
 - ii. In the case of liquid medication, the ward/clinical pharmacist must clarify the volume of liquid to be administered.
4. The frequency and time(s) of administration:
 - a. Medication prescribed as 'Once Only' doses must have a specific time for administration.
 - b. PRN medication must include the maximum dose to be given within 24 hours and the required observation time.
 - c. Where a medicine is prescribed on certain days of the week, the actual day/s are stated in the order (e.g., methotrexate on Wednesday only);
 - i. The days when the medicine is **not** to be administered **must** be crossed out in the administration section of the chart.
5. Addition instructions if required:
 - a. When to be taken with regard to food.
 - b. Transdermal patches removed or checked.
 - c. Specific instructions on taking/ using medicines.
6. Route of administration: oral, topical, sublingual, subcutaneous, intramuscular etc.
7. Start dates and times for administration.
8. Indication for medication as per the [Guidelines for the WA HMC](#).
9. Only approved abbreviations may be used. For detailed information refer to [Appendix 1 Acceptable Prescribing Terms and Abbreviations](#).
10. Corrections or amendments must be rewritten as a new order. The original order must be ceased and signed with initials and the date.
11. The patient's prescriber must review the medication chart daily. If unable to undertake daily review, risk assessments must be conducted to determine the frequency of ongoing chart review, based on the acuity or clinical risk of patients. Refer to [WA Health Medication Review Policy \(MP0104/19\)](#).

12. Pharmacists are permitted to clarify medication charts when necessary. Any significant clarifications must be discussed with the Medical Practitioner and documented in the patient's Integrated Progress Notes.

For more information refer to: [NMHS MH Medication: Formulary and Non-Formulary Prescribing Policy](#)

5.4 Telephone Orders

If a direction (order) to administer is provided other than in writing, such as verbally (face to face) or by telephone (or other electronic means) the Medicines and Poisons Legislation requires the **authorised health professional** receiving the direction to record the direction on the patient's medication chart and the chart must be signed by the prescriber within 24 hours of issuing the direction.

An authorised health professional is an RN or an EN

1. Telephone orders may be made where a single dose of medication is required, and the prescriber is not immediately available to write the order.
2. Telephone orders can only be taken by **authorised health professional**
3. A telephone order for a medication is valid provided only one dose is administered within a 24-hour period.
4. If the telephone medication order is not subsequently signed and the patient requires more medication, a further dose cannot be given until the Medical Practitioner reviews the patient.
5. The **authorised health professional** taking the telephone order must:
 - a. Repeat the order to the Medical Practitioner
 - b. Record the prescription on the "Telephone Orders" section of the WA HMC
 - c. A second **authorised health professional** must check the telephone order with the Medical Practitioner.
6. The telephone medication prescription must include:
 - a. Patient's name
 - b. Name of generic medication ([Appendix 4 Charting Medicines with Generic and/or Trade/Brand Names](#))
 - c. Time and date to be given
 - d. The dose to be administered
 - e. Frequency at which the medication is to be administered
 - f. Route of administration
 - g. Prescribing Medical Practitioner's name
 - h. Initials of the two (2) Nurses taking the order.
7. The **authorised health professional** taking the order must also record the telephone prescription in the patient's Integrated Progress Notes.
8. The final decision to accept a telephone medication prescription, rests with the authorised health professional.
9. Telephone orders **must not** be taken for S8 medications.



5.5 Administration of Medication

All nurses engaged in administering medication must be deemed qualified to administer medication. A Registered nurse **or** Enrolled Nurse may administer medications in accordance with legislation, policy, and best practice requirements.

1. Only **one nurse** is required to sign the medication chart except for:
 - a. Insulin, clozapine, S4Rs, S8s and cytotoxic medicines, which must always have two authorised staff involved in the administration and signing process.
2. Any medications administered 30 minutes or more either side of the prescribed time must have the actual time administered recorded in the medication chart above the staff member's initial. ([WA Guiding Principles for Timely Administration of Medications](#)).
3. Where two nurses are involved in administration, both nurses must check each facet of administration against the medication chart which will show the correct:
 - a. Patient ([NMHSMH Identification of Patients Policy](#))
 - b. Medication
 - c. Dose
 - d. Time
 - e. Route
 - f. Any ADR/allergies including details of the precise nature of the allergy, where possible.
4. When checking a medication prescription, nurses must always be aware of the following aspects of the medication before administration:
 - a. Generic name of the medication
 - b. Action and therapeutic purpose
 - c. Side effects
 - d. Adverse reactions
 - e. Interactions
 - f. Contraindications
 - g. Usual dosage
 - h. Frequency
 - i. Times for administration
 - j. Route of administration
 - k. Specific precautions.
5. Where there are patients of the same or similar name accommodated in the same ward a green caution label must be affixed to their medical record, medication chart and any other pertinent documentation to alert staff.
6. All medications, apart from the Nurse Initiated Medications listed in [Appendix 2](#) of this policy, must only be administered or used after a prescription has been written by a Medical Practitioner on the patient's medication chart, except for telephone orders.
7. An authorised health professional may administer medications listed in [Appendix 2](#) of this policy without prescription by a Medical Practitioner provided that the administration of the medication is recorded in the *"Once only, pre-medication and nurse/midwife initiated"*



medicines" section of the medication chart, dated and signed, and documented in the patient's integrated progress notes.

- a. The authorised health professional who initiates and administers the medication must also be the signatory on the medication chart.
 - b. The Medical Practitioner must be advised as soon as possible, and before any subsequent dose is given.
8. An authorised health professional (RN or EN) may administer Schedule 2, Schedule 3 and Schedule 4 medications in accordance with legislation, policy and best practice requirements.
 9. A RN will supervise a student nurse involved in the administration of medication and only the RN is authorised to sign the medication chart.
 10. The preparation of a medication dose including an injection must only occur immediately prior to administration.
 11. Intravenous medications must be drawn up and administered by a Medical Practitioner only.
 12. Partial tablets and refused doses including liquids of non-controlled medicines, must be disposed of in the sharps container. Medication must not be disposed of in sinks, toilets, or other general waste.
 13. For disposal of Controlled Medicines, please refer to the [NMHS Destruction, Discard & Disposal of S8 & S4R Medicines Policy](#) and [Procedure](#).
 14. Medication prescribed on the '*PRN Medication Chart for Agitation and Arousal*' should be administered in the order written. If there are compelling clinical reasons not to, the RN making the decision to go outside the order on the chart must document reasons why the order was not followed in the patient's integrated case notes.
 15. When patients are fasting, it is the responsibility of the nurse administering the medication to liaise with the Medical Practitioner to ascertain which medications should still be administered, if not indicated on the medication chart.
 16. A RN or EN may hand over dispensed medications to patients who are going on approved leave, provided the following occur:
 - a. The treating Medical Practitioner provides a script for pharmacy including medication(s), dosage(s), time of administration and the number of days or doses required on the day prior to leave.
 - b. Pharmacy dispenses the medications. Each medication will be appropriately packed and labelled indicating name of medication, dosage and time.
 - c. The nurse is to write "L" on the medication chart to record that the medication was given to the patient along with instructions for taking the medication as prescribed while on leave.
 - d. On return from leave, the Nurse must check for any unused medications:
 - i. Any unused medications must be returned to the pharmacy.
 - ii. If the patient has omitted doses, the Nurse must assess the patient for adverse effects of non-adherence with medication orders and notify the Medical Practitioner of non-adherence and effects.



-
- e. If the decision to grant daily leave is made outside of pharmacy opening hours the Medical Practitioner may dispense medications from the ward supply into boxes/bottles for the patient:
 - i. Medication must be labelled in accordance with the requirements of the [Western Australian Legislation - Medicines and Poisons Act 2014](#) and the [Western Australian Legislation - Medicines and Poisons Regulations 2016](#).
 - ii. A prescription for the medication dispensed from the ward supply must be written and sent to the pharmacy department. [Appendix 3 Medication Dispensed from Ward Stock](#).

17. Nurses are **NOT** permitted to dispense medication from ward supplies.

18. Blister Packs (e.g. Webster-paks®) may only be used by inpatients as part of the Medication Self Administration Program.

5.6 Refusal of Medication

Patients may have the right to refuse medications ([WA Mental Health Act 2014](#)). If a patient refuses any prescribed medications, this should be recorded on the medication chart.

The patient's Medical Practitioner should review the patient and indicate any action regarding further medications and treatment in the patient's integrated progress notes and on the medication chart if appropriate.

6. Error Management

Errors may occur at all steps related to prescribing, supply, dispensing, checking and administration. Any errors or near misses must be reported by completing a Datix CIMS. This form must be completed by the staff member reporting the incident.

The nominated person, carer, family member or personal support person must be notified in the case of a medication error ([WA Mental Health Act 2014](#)).

Any error in medication administration must result in immediate action to ensure the patient receives appropriate care and treatment.

- Errors should be reported immediately to the CNS/Shift Co-ordinator or equivalent and the Medical Practitioner. The CIMS reporting process must be followed including requesting a 'Third Party' comment must be requested from the NMHS MH Chief Pharmacist.

7. Consumer Medication Information (CMI)

In accordance with the [WA Medication Review Policy MP 0104/19](#), prescribers, nurses and pharmacists will offer up to date CMI, approved by the TGA or information endorsed by the WA Psychotropic Medication Group and discuss compliance and discontinuation issues when a medication is prescribed and reviewed.

1. This information will be accessible and readable for all patients, including the culturally and linguistically diverse community and Aboriginal People if possible.



-
2. This information will be accompanied by a verbal explanation of medication treatment options, benefits, associated risks and side-effects.
 3. A summary of the above actions will be documented in the patient's Integrated Progress Notes.
 4. If applicable, the information will also be provided to the patient's carer or family members.
 5. [The Choice and Medication Consumer Medication Information](#) website contains psychotropic medicines information in a range of different formats.

8. Nicotine Replacement Therapy (NRT)

Authorised nursing staff may initiate Nicotine Replacement Therapy (NRT) in accordance with the [NMHS MH Medication: Nurse Initiated Nicotine Replacement Therapy Policy](#).

Patients going on leave will be offered NRT to take with them to support the management of nicotine withdrawal while on leave.

- Patients on overnight leave will be prescribed NRT along with other leave medication.
- Where patients take leave from the ward for short periods, whether leaving the grounds or not, a RN can supply immediate release NRT from ward imprest stock.
- This will be documented on the NRT PRN Chart with the number of doses and the date and time given to the patient.

For more information refer to the [NMHS MH Smoking Care Guideline](#).

9. Medication for Hospital in The Home (HiTH) Patients

1. HiTH patients must use their own medication wherever possible.
2. A current medication profile must be maintained for each HiTH patient as defined in the [MHPHDS Adult Hospital in the Home Service, Model of Care](#). When a patient requires medication to be prescribed the prescription must be Pharmaceutical Benefits Scheme (PBS) compliant where possible. Prescriptions should be dispensed by the patient's community pharmacy. The patient co-payment for HiTH patients is covered by a voucher for any medication prescribed by HiTH excluding the patient's usual medication.
3. Where medication is administered by the HiTH nursing staff it must be blister packed:
 - a. Blister packs for administration by HiTH nursing staff may be provided by the patient's community pharmacy.
 - b. Blister packs should be kept at the patient's home, if appropriate. Alternatively, arrangements can be made for the patient's community pharmacy to store blister packs. Blister packs or any other medications should not be retained by the HiTH nursing staff except with permission of the NMHS MH Chief Pharmacist.
 - c. Nursing staff may observe self-medication practices where there are concerns regarding medication adherence.
 - d. Medication can be provided in tear-off blister packs for patients who are in transition to managing their own medication and can be left with one or more days' supply.
4. Where HiTH patients can manage their own medication and use blister packs the HiTH doctor or nursing staff must liaise with the patient's usual pharmacy for any changes in medication



-
5. HITH case managers support, prompt and if required monitor patients with self-medication and reinforce the need to take medication as prescribed. Issues identified are referred to HITH medical, nursing, and pharmacy staff for review and follow up as required.
 6. When a patient requires urgent additional medication, e.g. PRN psychotropics, the HiTH doctor should write a prescription and provide a voucher. This is dispensed by the patient's community pharmacy. If this happens after hours, the **HITH RN** should contact the duty doctor, and ask them to write a prescription.

10. Storage and Security of Medications

1. All medications must be stored in a secure locked medication cupboard/trolley in accordance with the WA Medicines and Poisons Act and WA Medicines and Poisons Regulations, manufacturer's storage recommendations and local policy:
 - a. S8 and S4R medications have additional storage requirements. Refer to NMHS MH Controlled (Schedule 8 and Schedule 4 Restricted) Medicines Policy.
 - b. Refrigerated pharmaceuticals must be stored in accordance with [NMHS MH Medication: Safe Use of Medication Refrigerators Policy](#).
2. The keys (or other access device) of the medication cupboard/trolley must be kept securely on the person of the nominated RN or EN.
3. The RN or EN must ensure that the cupboard/trolley is kept always locked.
4. Medications (apart from S4Rs and S8s) that are no longer required or have expired must be removed from ward stock and placed in the "Return to Pharmacy" box for collection by pharmacy staff.
5. Medications must not be transferred to another labelled or unlabelled container.
6. Substances from an unlabelled or indistinctly labelled container must not be used.
7. Any medication for a specific patient, (e.g. insulin or inhalers) must be labelled for the patient, either by pharmacy, or by the RN or EN opening the item who must affix a patient identification label to the primary container (e.g. inhaler device or insulin pen), ensuring expiry date and details remain visible.
8. Discontinued patient-specific medications should be removed from patient supplies and returned to pharmacy in a timely manner (e.g. upon the patient's discharge, discontinuation of the medicine, or during the next scheduled pharmacy rounds to patient care units) to prevent accidental administration or borrowing of the medication for another patient.

11. Patient's Own Medications (POMs)

1. All medications brought into a hospital/unit by a patient must be recorded on the Statement of Patients Own Medication sheet (SPOM) and stored according to local policy. This is to be completed, checked and signed by two authorised nurses.
2. The responsible Medical Practitioner must be informed as part of the Medication Reconciliation Process. These medications must not be administered to the patient except under exceptional circumstances and this must be authorised in writing by the Medical Practitioner or Pharmacist. Refer to local procedures on POM.

3. On discharge the POM should be returned where appropriate. The Medical Practitioner should be consulted, and the decision taken documented in the patient's Integrated Progress Notes.
4. Any unclaimed medications will be destroyed by the pharmacy department two weeks after discharge.

[NMHS Management of Patients' Own Schedule 8\(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#)

[MH Medication: Patient's Own Medication Policy and Procedure](#)

12. Controlled Medicines

S8 medicines may also be referred to as controlled substances, drugs of dependence or drugs of addiction.

S4R medicines are also controlled substances and are liable to be abused and may cause dependence. These medicines require special storage and record keeping procedures for accountability.

For information on storage, administration and record keeping please refer to [WA Medicines Handling Policy \(MP 0139/20\)](#) and [NMHS Medicines Management Policies](#).

13. Legislative Requirements

The relevant legislation concerning medication control in WA is contained in the [Western Australian Legislation - Medicines and Poisons Act 2014](#) and the [Western Australian Legislation - Medicines and Poisons Regulations 2016](#)

All nursing and medical staff are advised to familiarise themselves with these documents.

14. Compliance and Evaluation

- This policy will be audited by NMHS MH Pharmacy. Compliance with medication prescribing and administration and storage requirements will be reported and monitored via the Mental Health Management Committee
- Medication errors reported via Datix CIMS will be audited and reported to the NMHS MH Drugs and Therapeutics Committee and Mental Health Management Committee.

Related internal policies, procedures and guidelines

[NMHS Administration of Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy and Procedure](#)

[NMHS MH Patient's Own Medications Policy and Procedure](#)

[NMHS MH Clinical Record Documentation Policy](#)

[NMHS MH Medication: Nurse Initiated Nicotine Replacement Therapy](#)

[NMHS MH Medication: Safe Administration of Liquid Medications](#)
[NMHS MH Medication: Lorazepam – Storage and Management](#)
[NMHS MH Medication: Formulary and Non-Formulary Prescribing Policy](#)
[NMHS Medication: Management of Patient's Own Schedule 8 \(S8\) and Schedule 4 Restricted \(S4R\) Medicines Policy](#)
[NMHS MH Medication: After-Hours Supply Policy](#)
[NMHS MH Medication: Reporting of Medication Discrepancies in Inpatient and Community Services Policy](#)
[NMHS MH Medication Reconciliation Policy](#)
[NMHS Recognising and Responding to Acute Deterioration Procedure](#)
[NMHS MH Identification of a Patient Procedure](#)

References

[WA Medicines Handling Policy MP 0139/20](#)
[WA Medication Chart Policy MP 0078/18](#)
[Guidelines for the use of the Adult WA Hospital Medication Chart \(WA HMC\)](#)
[WA Medication Review Policy MP 0104/19](#)
[Western Australian Legislation - Medicines and Poisons Act 2014](#)
[Western Australian Legislation - Medicines and Poisons Regulations 2016](#)
[Reporting of Schedule 4 Restricted and Schedule 8 Medicines Discrepancies Policy MP 0103/19](#)
[Choice and Medication Consumer Information](#)
[Statewide Medicines Formulary Policy MP 0077/18](#)
[WA Guiding Principles for Timely Administration of Medications](#)
[NMHS Medicines Management Policies](#)
[WA Clinical Alert \(MedAlert\) Policy](#)
[WA Health Credentialing and Defining the Scope of Clinical Practice Policy MP 0084/18](#)
[NMHS Patient Identification and Procedure Matching Policy](#)

Useful resources

[Universal Ward Imprest List](#)

Policy Sponsor	NMHS MH Chief Pharmacist				
Policy Contact	NMHS MH Chief Pharmacist				
Date First Issued:	09/05/2012	Last Reviewed:	16/01/2024	Review Date:	05/10/2026
Approved by:	NMHS MH Drugs and Therapeutics Committee			Date:	05/12/2023
Endorsed by:	MHPHDS Executive Director			Date:	23/01/2024
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers		☒  Std 4: Medication Safety		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☒ Std 3: Consumer and Carer Participation		☒ Std 8: Governance, leadership and management ☒ Std 10: Delivery of Care		
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	Version	Author	Notes
18/06/2019	V2	MHEG	Endorsed out of session via email
24/06/2019	V2	Executive Director	Endorsed via Submission form
24/06/2019	V2.1	Chief Pharmacist, NMHS MH	Major Amendment: Addition of statement on provision of consumer medicines information, updated template and reformatting, updated hyperlinks, update to Nurse Initiated NRT provisions in keeping with the NMHS MH AW Medication: Nurse Initiated Nicotine Replacement Therapy Policy. No change to scheduled review date.
23/05/2022	V3	NMHS MH Chief Pharmacist	Scheduled Review, updated template, links; Information on controlled medications (S8 & S4Rs) removed, several WA and NMHS policies & procedures..
09/02/2023	V3.1	Audit and Policy Officer	Update of Hyperlinks
16/10/2023	V4	NMHS MH Medication Safety Pharmacy	Amendments to remove barriers to EN scope of practice
16/01/2024	V4.1	Chief Pharmacist, NMHS MH	Update nurse initiation appendix 2; added paliperidone Hafyera® to appendix 4

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative ISD1227.

This document can be made available in alternative formats on request for a person with a disability.

© North Metropolitan Health Service 2023

Copyright to this material is vested in the State of Western Australia unless otherwise indicated. Apart from any fair dealing for the purposes of private study, research, criticism or review, as permitted under the provisions of the *Copyright Act 1968*, no part may be reproduced or re-used for any purposes whatsoever without written permission of the State of Western Australia.

Appendix 1: Acceptable Prescribing Terms and Abbreviations

Acceptable Prescribing Terms and Abbreviations

Intended meaning	Acceptable term or abbreviation	Intended meaning	Acceptable term or abbreviation
Dose frequency or timing		Route of administration	
(in the) morning	morning, mane	ear	ear(s) specify left, right or both
(at) midday	midday	epidural	epidural
(at) night	night, nocte	eye	eye(s) specify left, right or both
twice a day	bd	inhale, inhalation	Inhale, inhalation
three times a day	tds	intraarticular	intraarticular
four times a day	qid	intramuscular	IM
every 4 hours	every 4 hrs, 4 hourly, 4 hrly	intranasal	intranasal
every 6 hours	every 6 hrs, 6 hourly, 6 hrly	intrathecal	intrathecal
every 8 hours	every 8 hrs, 8 hourly, 8 hrly	intravenous	IV
once a week	once a week and specify the day in full	irrigation	irrigation
three times a week	three times a week and specify the days in full	left	left
when required	prn	naso-gastric	NG
immediately	stat	nebulised	NEB
before food	before food	oral	PO
after food	after food	percutaneous enteral gastrostomy	PEG
with food	with food	per rectum	PR
Dose forms		per vagina	PV
capsule	cap	peripherally inserted central catheter	PICC
cream	cream	right	right
drops	drops	subcutaneous	subcut
injection	inj	sublingual	subling
metered dose inhaler	metered dose inhaler, inhaler, MDI	topical	topical
mixture	mixture	Units of measure and concentration	
ointment	ointment, oint	gram(s)	g
patient controlled analgesia	PCA	litre(s)	L
pessary	pess	milligram(s)	mg
powder	powder	millilitre(s)	mL
suppository	supp	microgram(s)	microgram, microg
tablet	tablet, tab	millimole	mmol
		international unit(s)	international unit(s)
		unit(s)	unit(s)
		percentage	%



Government of Western Australia
Department of Health



CCM01016 JAN17 © Department of Health 2017

better health = better care = better value

Appendix 2: Medications which may be administered by a nurse without a prescription

Any medications administered from this appendix must be recorded in the appropriate section of the WA HMC and in the patient's Integrated Progress Notes.

ACTION	MEDICATION
Analgesia	Paracetamol
Laxatives	Macrogol (e.g. Movicol®) Docusate with Senna (e.g. Coloxyl with Senna®) Glycerol Suppositories Bisacodyl Suppositories
Cough relief preparations	Bromhexine Liquid Throat lozenges
Liquid Antacids <i>(do not give within 2 hours of other medications)</i>	Gastrogel®/ Mylanta®
Nicotine Replacement Therapy (NRT) See: Nurse Initiated Nicotine Replacement Therapy	Nicotine Lozenge 2mg/4mg Nicotine Inhalator 15mg Nicotine Spray 1mg/dose Nicotine Patch 21mg/24hrs
Topical preparations	Methyl salicylate cream (eg. Metsal®) Bepanthen® cream Paraffin based ointment e.g. Dermeze® Glycerol 10% in Cetomacrogol Cream (Sorbolene cream) Lanolin ointment Aqueous cream Dimeticone Cream and Lotion Bonjela® gel

Appendix 3: Medication Dispensed from Ward Stock

Aim

To fulfil the legal requirements of the **Medicines and Poisons Regulations 2016**, all medication dispensed from ward stock must follow the following procedure.

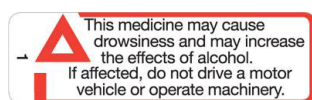
Medication must only be provided from ward stock outside the Pharmacy Department opening hours. This should only occur for brief periods of unexpected leave. If discharge medication is required, the doctor will provide the patient with a prescription and voucher for dispensing at a Community Pharmacy.

Procedure

1. All the following steps **must be** completed by the **Doctor**.
2. Use the designated empty medication containers. A paper bag or an envelope **must not** be used as a container.
3. Complete the designated blank label. The label must be clearly completed with the following information:
 - medication name
 - medication strength
 - medication quantity
 - directions for use
 - patient name and URMN
 - date of dispensing
 - prescriber name
4. Each individual medication must be dispensed with a separate container with the completed label.
5. **A prescription must be generated and sent to the pharmacy department for legal record keeping. The prescription must be marked as “dispensed from ward.”**
6. Any medications appearing in Appendix K of The Poisons Standard (the SUSMP) is required by law to be labelled with a sedation warning. Refer to Appendix K.
7. Pharmacy must be contacted when supply of containers and labels are required.

Appendix K – medicines required to be labelled with a sedation warning

Medications in bold are those commonly used at NMHS Mental Health. Refer to Appendix K of the SUSMP for the most current list of medications.



Alimemazine	Dextromoramide	Mazindol	Prazepam
Alprazolam	Dextropropoxyphene	Mebhydrolin	Pregabalin
Amisulpride	Diazepam	Meclozine	Prochlorperazine
Amitriptyline	Difenoxin	Medazepam	Promazine
Amobarbital	Dihydrocodeine	Meprobamate	Promethazine
Aripiprazole	Dimenhydrinate	Mepyramine	Protriptyline
Asenapine	Dimethindene	Mercaptamine	Quetiapine
Azatadine	Diphenhydramine	Methadone	Retigabine
Baclofen	Diphenoxylate	Methdilazine	Risankizumab
Benzatropine	Diphenylpraline	Methocarbamol	Risperidone
Brexipiprazole	Dosulepin	Methylphenobarbital	Rotigotine
Brivaracetam	Doxepin	Mianserin	Rufinamide
Bromazepam	Doxylamine	Midazolam	Rupatadine
Brompheniramine	Dronabinol	Mirtazapine	Safinamide
Bucizine	Droperidol	Morphine	Secbutobarbital
Buprenorphine	Duloxetine	Nabiximols	Secobarbital
Butobarbital	Esketamine	Nalbuphine	Seletracetam
Cannabis	Ethylmorphine	Nitrazepam	Sodium Oxybate
Cetirizine	Fenfluramine	Normethadone	Stiripentipol
Chloral hydrate	Fentanyl	Nortriptyline	Suvorexant
Chlordiazepoxide	Flunitrazepam	Olanzapine	Tapentadol
Chlormethiazole	Flupentixol	Opium	Temazepam
Chlorphenamine	Fluphenazine	Oxazepam	Tetrahydrocannabinols
Chlorpromazine	Flurazepam	Oxycodone	Thenylidamine
Clemastine	Gabapentin	Paliperidone	Thiethylperazine
Clomipramine	Gemcitabine	Papaveretum	Thiopropazate
Clonazepam	Glutethimide	Pentazocine	Thioridazine
Clonidine	Guanfacine	Pentobarbital	Tramadol
Clorazepate	Haloperidol	Perampanel	Tranlycypromine
Clozapine	Hydrocodone	Periciazine	Trifluoperazine
Codeine	Hydromorphone	Perphenazine	Trimipramine
Cyclizine	Hydroxyzine	Pethidine	Tripolidine
Cyclobarbitol	Imipramine	Phenelzine	Ziprasidone
Cycloserine	Lamotrigine	Pheniramine	Zolpidem
Cyproheptadine	Levetiracetam	Phenoperidine	Zonisamide
Dantrolene	Levocabastine	Phenyltoloxamine	Zopiclone
Desipramine	Levocetirizine	Pholcodine	
Deutetrabazine	Lorazepam	Pimozide	
Dexchlorphenamine	Lurasidone	Pizotifen	

Appendix 4: Charting Medicines with Generic &/or Trade/ Brand Name

The generic name of medicine should be charted on medication charts as per the WA Hospital Medication Chart guidelines 2017ⁱ.

High Risk Medications:

Insulinⁱⁱ: Full trade name e.g. Humulin® 30/70 not just Humulin® *include form of insulin device*

Warfarinⁱⁱⁱ: Different brands of warfarin are not bioequivalent and have different tablet dose sizes. Trade and generic names must be written e.g. warfarin (Marevan® or Coumadin®)

Opioids^{iv}: use generic and brand name plus form (SR, XR, CR or MR never IR)

- Oxycodone e.g. OxyNorm®, OxyContin®; combinations: oxycodone/naloxone (Targin®)
- Morphine e.g. MS Contin®
- Hydromorphone preparations e.g. Jurnista®
- Buprenorphine e.g. Suboxone®, Subutex®, Temgesic®

Chart with both GENERIC (first) and TRADE (brackets) names on Medication Charts^v

Oral Iron Supplements: e.g. ferrous sulfate 325mg/ascorbic acid 500mg (Ferrograd C®)

Calcium containing products: e.g. calcium carbonate (Calsup®), calcium/ colecalciferol (Cal-600+D®)

Latanoprost: latanoprost/ timolol (Xalacom®) to distinguish from latanoprost (Xalatan®)

Inhalers: e.g. fluticasone/salmeterol (Seretide®), budesonide/ formoterol (Symbicort®) - *strength & delivery device should be stated*

Combination medications acceptable for TRADE name prescribing on medication charts

Lubricant eye drops: e.g. Polytears®, Refresh®

Parkinson medication: e.g. Madopar®, Sinemet®, Stalevo® - *strength & delivery system should be stated*

Laxatives: e.g. Coloxyl with Senna®, Movicol®, LaxaCon®

Oral Contraceptives: combination; brand equivalent e.g. Levlen ED®, Trifeme®

Others: Metamucil®, Benefiber®, Sofradex®

Long Acting Intramuscular (LAI) Depot

Paliperidone: Paliperidone Trinza®; Paliperidone Hafyera® (*for 3 or 6 monthly LAIs ONLY*)

Medicines that should NOT be charted with trade names

- Antibiotics (may not be recognised as penicillin, if brand names are used)
- Paracetamol combinations
- Antihypertensive combinations

Note: *This is not a complete list please speak to your pharmacist for any queries.*

ⁱ WA Hospital Medication Chart Guidelines 2017 <http://ww2.health.wa.gov.au/-/media/Files/Corporate/general%20documents/Quality/PDF/Medication%20chart/WA-HMC-User-guide.pdf>

ⁱⁱ WA High Risk Policy 2020 <https://ww2.health.wa.gov.au/-/media/Files/Corporate/Policy-Frameworks/Clinical-Governance-Safety-and-Quality/Policy/High-Risk-Medication-Policy/High-Risk-Medication-Policy.pdf>

ⁱⁱⁱ MIMs PI information

^{iv} WA MSG Safe use of opioids in hospital <https://ww2.health.wa.gov.au/-/media/Files/Corporate/general-documents/safety/PDF/Medication-safety-resources/Safe-Use-of-Opioids.pdf>

^v OPH 'Charting of Generic and Trade names' 2014